Reducing Chronic Absenteeism in Washington, D.C. Public Schools:
A Comprehensive Policy Proposal for Mayoral Action
By:

DETAILED OUTLINE

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1. INTRODUCTION

1.1 Opening Hook
- Start with compelling statistic: 39.2% CA rate = 32,384 students
- Brief comparison to national average (30%)
- Human impact framing: what this means for individual students and families

1.2 Problem Statement
- Define chronic absenteeism (missing ≥10% of school days)
- Establish why this is a crisis requiring urgent mayoral action
- Preview the multi-dimensional nature of the problem

1.3 Purpose and Scope
- State the paper's objective: propose comprehensive policy framework
- Identify the audience: Mayor's Office
- Outline the paper's structure

1.4 Thesis Statement
- Clear statement: A comprehensive five-pillar framework addressing family engagement, early warning systems, community hubs, health services, and accountability mechanisms offers the most effective, efficient, and equitable approach to reducing chronic absenteeism in D.C. public schools.

Problem Analysis

Trends and Disparities in D.C.

Washington D.C. is facing a chronic absenteeism crisis. In the 2022-23 academic year, 40% of K-12 students were chronically absent, well above the national average of 28% (Office of the State Superintendent of Education, 2024). Absenteeism is concentrated at the high school level (56% chronic absenteeism), and highest in the ninth grade (65.25), suggesting that the

transition into high school is a key time for intervention (Coffin & Mason, 2025). Public school students are more likely to be absent than charter school students (ibid).

Chronic absenteeism is experienced along stark equity lines. Students who are unhoused, economically disadvantaged, and/or TANF/SNAP eligible are disproportionately affected (Office of the State Superintendent of Education, 2024). Wards 7 and 8, home to the highest concentrations of students living in poverty, have the highest rates of chronic absenteeism (Office of the Deputy Mayor for Education, n.d.). The crisis also has racial dimensions: Black students are 3.9 times more likely, and Hispanic students 2.8 times more likely, to be chronically absent than their White peers (Office of the State Superintendent of Education, 2024).

Post-pandemic data analysis reveals shifts in chronic absenteeism patterns. Eighth grade saw the largest increase in chronic absenteeism, by 21 percentage points (Coffin and Mason, 2025). Female students are now more likely to be chronically absent, a reverse of pre-COVID patterns. In high schools, absenteeism is tied less to student behavior and more to individual or family circumstances (ibid). These trends suggest a need for greater student and parent-based interventions in middle school with particular attention to female students.

Root Causes of Chronic Absenteeism

The drivers of chronic absenteeism reflect broader social and economic inequities. Economic disadvantage is one of the most powerful predictors of future absenteeism (Coffin and Mason, 2025). Post-pandemic listening sessions reveal significant disengagement and perceptual obstacles, with students reporting mental health difficulties and a feeling that in-person learning is less essential (Mason, 2024). Students and families also commonly cite transportation safety concerns and illness as barriers (Coffin & Mason, 2025).

Perhaps the most notable facet of chronic absenteeism is its persistence. According to the Office of the State Superintendent of Education (2024), students chronically absent in pre-K are

41.5 percentage points more likely to be chronically absent in kindergarten. This holds in later grades, as well: 8th grade absenteeism is the strongest predictor of 9th grade chronic absenteeism (Coffin & Mason, 2025). This dynamic underscores the importance of early warning systems, proactive communication, and outreach to moderately absent students before patterns solidify.

Consequences of Chronic Absenteeism

Chronic absenteeism carries academic, social, and long-term economic consequences. Students who miss substantial instructional time score lower on standardized tests and are more likely to repeat a grade (Gottfried, 2014; Santibañez & Guarino, 2021; Humm Patnode et al., 2018). Absenteeism also undermines socioemotional development, including belonging and motivation (ibid). Long-term, chronically absent students are less likely to graduate high school or complete postsecondary education and face reduced lifetime earnings (Ansari, Hofkens & Pianta, 2020; Jacob & Lovett, 2017). For schools, widespread absenteeism increases remediation needs, strains staffing, and can damage community trust (Lieberman, 2024).

Current Policy Landscape

In the post-pandemic period, D.C. has prioritized absenteeism reduction. The Bowser administration has committed to reducing chronic absenteeism by 50% in 5 years (Attendance Works, n.d.). Efforts are spearheaded by a new "Every Day Counts!" taskforce, which has created an attendance website resource hub, launched a public messaging campaign, and began an attendance pledge drive. Policymakers are also reshaping the legal framework to emphasize supportive over punitive responses. The 2024 Truancy Reduction for Student Success Act created a pilot program to refer students with 15 unexcused full school day absences to human services rather than the juvenile probation agency (Council of the District of Columbia, 2024). In its first year, participating students showed a 50% increase in attendance and improved behavioral functioning; the pilot is now being extended and expanded for a second year (D.C.

Department of Human Services, 2025). Furthering these efforts, Councilmember Parker has introduced the Improving School Attendance Outcomes Amendment Act of 2025, which would strengthen early intervention efforts based on the pilot's findings.

DC has also increased its support for programs targeting core attendance barriers. The Show Up, Stand Out program creates individualized attendance plans alongside parents of K-8th grade children with 5 or more absences (Office of Victim Services and Justice Grants, 2022). Kids Ride Free and Safe Passage address transportation cost and safety barriers (District Department of Transportation, n.d.; DC.Gov, n.d.). Local partnerships such as the one with Children's National Hospital address health barriers to educational attendance (Children's National, n.d.).

Despite these investments, significant gaps remain. Data collection and reporting remain fragmented across agencies (AJE-DC, 2025). Stronger coordination amongst CFSA, DHS, OSSE, and DCPS is necessary to generate more timely interventions. A lack of coordination between families, schools, and healthcare providers leaves vulnerable students without a holistic care system (ibid). Resource constraints further limit intervention capacity. These challenges point to the need for collaborative, data-driven, and transparent policy strategies.

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3. LITERATURE REVIEW & EVIDENCE BASE

3.1 Theoretical Framework
  3.1.1 Ecological Systems Theory
    - How factors at multiple levels (individual, family, school, community) interact to influence attendance
    - Justification for multi-pillar approach

  3.1.2 Response to Intervention (RTI) / MTSS Framework
    - Tiered support model for addressing student needs
    - Application to attendance intervention

3.2 Evidence on Effective Interventions

3.2.1 Family Engagement Interventions

Chicago's FACE Initiative

- Program description
- Implementation approach
- Outcomes achieved
- Lessons for D.C.

Connecticut's LEAP Program

- Program description
- Outcomes achieved
- Transferable elements

Pittsburgh's Connect-Text (Two-Way Texting)

- Smythe-Leistico & Page (2018) findings
- Effect sizes and cost-effectiveness
- Applicability to D.C. context

3.2.2 Early Warning Systems and Data-Driven Interventions

New York City's EWS Approach

- System design and implementation
- How data is used for intervention
- Outcomes achieved

Research on EWS Effectiveness

- Cite meta-analyses or systematic reviews
- Key features of effective EWS

MTSS Implementation Research

- Evidence on tiered support effectiveness
- Implementation fidelity factors

3.2.3 Community-Based and Wraparound Services

Community Schools Model

- Evidence on integrated services approach
- Relevance to Attendance Action Hubs concept

Cross-Agency Coordination Models

- Examples from other jurisdictions
- Evidence on service integration effectiveness

3.2.4 School-Based Health Services

Lim et al. (2023) - SBHCs and Attendance

- Study findings on health center impact
- Mechanisms linking health services to attendance

Broader SBHC Research
- Meta-analyses on SBHC effectiveness
- Mental health services and attendance
- Cost-effectiveness evidence

3.2.5 Accountability and Recognition Mechanisms
Rhode Island's Statewide Strategy
- Cohen (2024) - Future Ed report
- Leaderboard and recognition approach
- Statewide coordination lessons
- Outcomes achieved

Research on Positive Accountability
- Evidence on recognition vs. punitive approaches
- School improvement and motivation research

3.3 Synthesis: Why an Integrated Approach?
3.3.1 Limitations of Single-Intervention Approaches
- Research showing modest effects of isolated interventions
- Students face multiple barriers requiring multiple solutions

3.3.2 Evidence for Comprehensive Models
- Studies comparing integrated vs. isolated approaches
- Community schools and wraparound services research

3.3.3 Application to D.C. Context
- How evidence translates to D.C.'s specific circumstances
- Unique advantages of D.C.'s governance structure

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4. POLICY PROPOSAL (3 pages)

4.1 Overview of Five-Pillar Framework
- Visual diagram of the five pillars and how they interact
- Theory of change: How each pillar addresses specific root causes
- Office of Equitable Attendance as coordinating body

4.2 Pillar 1: Tiered Family Engagement Corps
4.2.1 Program Design
- Staffing model: Number of liaisons per school
- Qualifications: Cultural competency, language skills, community ties

- Training: Content and duration

4.2.2 Operational Approach
- Tiered engagement based on absence severity
- Communication methods (texting, calls, home visits)
- Coordination with school staff and community services
- Documentation and data tracking

4.2.3 Evidence Connection
- How this draws on Chicago, Connecticut, Pittsburgh models
- Expected outcomes based on evidence

4.3 Pillar 2: Early Warning Systems (EWS) and MTSS

4.3.1 EWS Design
- Data inputs: Attendance, academics, behavior, other indicators
- Algorithm/thresholds for identifying at-risk students
- Dashboard and reporting features
- Integration with existing DCPS systems

4.3.2 MTSS Implementation
- Tier 1: Universal supports for all students
- Tier 2: Targeted interventions for at-risk students
- Tier 3: Intensive supports for chronically absent students
- Team structure and decision-making processes

4.3.3 Evidence Connection
- How this draws on NYC and other models
- Expected outcomes based on research

4.4 Pillar 3: Community-Based Attendance Action Hubs

4.4.1 Hub Design
- Number and geographic distribution of hubs
- Physical space and accessibility requirements
- Hours of operation

4.4.2 Services and Staffing
- Agency partners: DHS, CFSA, DHCD, DCPS, CBOs
- Services offered: Housing, food, childcare, transportation, etc.
- Staffing model: Hub director, case managers, agency liaisons

4.4.3 Coordination Mechanisms
- Referral pathways from schools to hubs
- Case management and follow-up processes
- Data sharing between agencies

4.4.4 Evidence Connection
- How this draws on community schools and wraparound models
- Expected outcomes

4.5 Pillar 4: School-Based Health Centers and Mobile Units

4.5.1 SBHC Expansion
- Criteria for school selection (CA rate ≥30%)
- Services offered: Primary care, mental health, dental, vision
- Staffing model: Nurses, counselors, providers
- Insurance and payment model

4.5.2 Mobile Health Units
- Number of units and deployment strategy
- Services offered
- Scheduling and coordination with schools

4.5.3 Health-Attendance Connection
- How health services reduce attendance barriers
- Protocols for connecting health and attendance interventions

4.5.4 Evidence Connection
- Lim et al. (2023) and other SBHC research
- Expected outcomes

4.6 Pillar 5: Student Attendance Leaderboard

4.6.1 Design
- Metrics: Overall CA rate, improvement rate, equity-adjusted measures
- Public-facing dashboard features
- Update frequency

4.6.2 Recognition Program
- Categories of recognition
- Incentives for improving schools
- Best practice sharing mechanisms

4.6.3 Evidence Connection
- Rhode Island model
- Research on positive accountability

4.7 Office of Equitable Attendance

4.7.1 Structure and Authority
- Placement within OSSE
- Leadership and staffing

- Cross-agency coordination authority

4.7.2 Functions
- Overall coordination of five pillars
- Data integration and analysis
- Monitoring and evaluation
- Stakeholder engagement
- Reporting to Mayor and City Council

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5. IMPLEMENTATION STRATEGY (2.5 pages)

5.1 Phased Implementation Approach

5.1.1 Phase 1: Pilot Launch (Year 1, Months 1-12)

Months 1-3: Foundation
- Executive order establishing Office of Equitable Attendance
- Hire director and core staff
- Finalize pilot school selection (12 schools)
- Develop detailed implementation plans
- Initiate procurement processes

Months 3-6: Infrastructure Building
- Recruit and hire Family Engagement Corps
- Begin liaison training program
- Procure EWS technology platform
- Secure facilities for initial Attendance Hubs
- Begin SBHC expansion planning

Months 6-9: Soft Launch
- Deploy liaisons to pilot schools
- Pilot EWS in select schools
- Open first Attendance Hub(s)
- Begin mobile health unit operations

Months 9-12: Full Pilot Operations
- All five pillars operational in pilot schools
- Launch public leaderboard with pilot schools
- Begin data collection for evaluation
- Conduct mid-year assessment and adjustments

5.1.2 Phase 2: Evaluation and Expansion (Years 2-3)

Year 2
- Comprehensive evaluation of pilot results

- Refine program models based on lessons learned
- Expand to next cohort of schools (20-30 schools)
- Scale up Attendance Hubs and health services

Year 3
- Continue expansion (40-60 schools total)
- Further refinement based on ongoing evaluation
- Prepare for districtwide rollout
- Build sustainability into budget and operations

5.1.3 Phase 3: Districtwide Implementation (Years 4-5)
- Systematic expansion to all 117 schools
- Full staffing of Family Engagement Corps
- Attendance Hubs covering all D.C. neighborhoods
- Health services access at or near all high-need schools
- Mature evaluation and continuous improvement systems

5.2 Key Implementation Milestones
- Table of major milestones with target dates
- Responsible parties for each milestone
- Success criteria for each milestone

5.3 Risk Assessment and Mitigation
5.3.1 Implementation Risks
Cross-Agency Coordination Challenges
- Risk description
- Likelihood and impact
- Mitigation strategies

Technology Implementation Delays
- Risk description
- Likelihood and impact
- Mitigation strategies

Staffing and Capacity Constraints
- Risk description
- Likelihood and impact
- Mitigation strategies

Community Resistance or Mistrust
- Risk description
- Likelihood and impact
- Mitigation strategies

5.3.2 External Risks

Funding Uncertainty
- Federal grant changes
- Budget pressures
- Mitigation strategies

Political Changes
- Leadership transitions
- Shifting priorities
- Mitigation strategies

5.4 Monitoring and Evaluation Plan

5.4.1 Evaluation Design
- Quasi-experimental design: Pilot vs. matched comparison schools
- Interrupted time series analysis
- Process evaluation alongside outcome evaluation

5.4.2 Outcome Measures

Primary Outcome
- Chronic absenteeism rate (target: from 39.2% to X%)

Secondary Outcomes
- Overall attendance rate
- Equity gap reductions (by race, income, disability)
- Student academic outcomes (test scores, grades, promotion)
- Socioemotional outcomes

5.4.3 Process Measures
- Implementation fidelity for each pillar
- Service utilization (liaisons, hubs, health centers)
- Stakeholder satisfaction
- Cost per student served

5.4.4 Data Collection and Reporting
- Data sources and collection frequency
- Real-time dashboards for operational monitoring
- Quarterly reports to leadership
- Annual comprehensive evaluation reports
- Public reporting via leaderboard

5.4.5 Evaluation Partnership
- Independent evaluator (university or research organization)
- IRB and ethical considerations

- Dissemination plan for findings

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6. COST ANALYSIS & FUNDING STRATEGY (1.5 pages)

6.1 Detailed Cost Estimates

6.1.1 Five-Year Budget Overview

Year 1 (Pilot - 12 schools): $X million
Year 2 (Expansion - 30 schools): $X million
Year 3 (Expansion - 60 schools): $X million
Year 4 (Full - 90 schools): $X million
Year 5 (Full - 117 schools): $X million

Total 5-Year Investment: $X million

6.1.2 Cost Breakdown by Pillar (at full implementation)

Pillar 1: Family Engagement Corps
- Personnel (X liaisons × $X salary + benefits): $X million
- Training and professional development: $X
- Communications and technology: $X
- Subtotal: $X million

Pillar 2: EWS and MTSS
- Technology platform (procurement + licensing): $X
- Data integration and infrastructure: $X
- Staff training: $X
- Technical support: $X
- Subtotal: $X million

Pillar 3: Attendance Action Hubs
- Facilities (lease/renovation for X hubs): $X million
- Personnel (hub directors, case managers): $X million
- Operating costs: $X
- Subtotal: $X million

Pillar 4: Health Services
- SBHC construction/renovation (X schools): $X million
- Mobile health units (X units): $X million
- Healthcare personnel: $X million
- Equipment and supplies: $X
- Subtotal: $X million

Pillar 5: Leaderboard

- Platform development: $X
- Maintenance and communications: $X
- Recognition program: $X
- Subtotal: $X

Office of Equitable Attendance
- Personnel (director, managers, analysts): $X million
- Operations: $X
- Subtotal: $X million

6.1.3 Cost Comparison with Alternatives
- Option B (Pillars 1 & 2 only): $X million (5-year)
- Option C (Status quo plus): $X million (5-year)
- Analysis of cost-effectiveness differences

6.2 Funding Strategy
6.2.1 District Budget Allocation
- Proposed general fund allocation: $X million over 5 years
- Budget justification: ROI from reduced dropout, protected enrollment funding
- Alignment with mayoral priorities

6.1.2 Cost Breakdown by Pillar (at full implementation)
Pillar 1: Family Engagement Corps
• Personnel (1 liaison per school × $X salary + benefits): $X million
● Expected $92,960.59 in salary ($63,169.90)
1
, benefits ($21,579.53), and
administrative add-ons ($8,211.16) in base year (FY27), expected average of
+3% appreciation/year
2
Annualized formula: (Schools * Base personnel cost * appreciation rate ^ years since base)
● Year 1 (Pilot - 12 schools - FY27): 12 * 92,960.59*1.03^0
● Year 2 (Expansion - 30 schools - FY28): 30 * 92,960.59*1.03^1
● Year 3 (Expansion - 60 schools - FY29): 60 * 92,960.59*1.03^2
● Year 4 (Full - 90 schools - FY30): 90 * 92,960.59*1.03^3
● Year 5 (Full - 117 schools - FY31): 117 * 92,960.59*1.03^4 = $12,241,471.67
● 5-year total: $31,289,043.58
• Approach specifics:
● Onboarding and approach-specific training: 4 weeks = cost * (4/52) in hired time
ahead of school year
● Year 1 (Pilot - 12 schools - FY27): 12 * 92,960.59*1.03^0 * (4/52) = $85,809.78
● Year 2 (Expansion - 18 new schools - FY28): 18 * 92,960.59*1.03^1 * (4/52)
● Year 3 (Expansion - 30 new schools - FY29): 30 * 92,960.59*1.03^2 * (4/52)
● Year 4 (Full - 30 new schools - FY30): 30 * 92,960.59*1.03^3 * (4/52)
● Year 5 (Full - 27 new schools - FY31): 27 * 92,960.59*1.03^4 * (4/52)

● 5-year total: $897,695.73
• Communications and technology
● $10.00/worker/month
3 = 10 * workers * 12
● Year 1 (Pilot - 12 schools): 10 * 12 * 12 = $1,440

Year 2 (Expansion - 30 schools): 10 * 30 * 12 = $4,320
● Year 3 (Expansion - 60 schools): 10 * 60 * 12 = $8,640
● Year 4 (Full - 90 schools): 10 * 90 * 12 = $12,960
● Year 5 (Full - 117 schools): 10 * 117 * 12 = $16,848
● 5-year total: $44,496.00
• Subtotal: $32,231,175.31

IIRC the best ways for funding are (1) ESSA Title I which cannot fully fund on its own given current levels (2) restaffing within DCPS system so payrolls are held over instead of added, (3) federal funds, details TBD, and (4) interagency cuts re DC budget at large

From Jeffrey
What I remember of my work is mainly about the costimates. It would be nice if someone could corroborate how much money we actually have to work with. sources are ESSA sections 1–4 I think, TANF, and Medicaid (but idk what set asides within medicaid actually apply).

TANF funding

- TANF funding perhaps could help fund the community action hubs - used for the administrative costs of the hubs and the cross-agency teams that coordinate services to
- Direct Support/Services: Funds could be used for the direct services provided at the hubs, particularly non-recurrent, short-term benefits (NRSTs). NRSTs are designed to deal with a specific crisis (like an eviction notice or a temporary lack of food) and are not intended to meet ongoing needs.
    - Examples of TANF-eligible NRSTs: Paying for emergency housing assistance, utility arrears, emergency transportation (e.g., to a doctor's appointment or work), or a one-time financial aid payment to address a food crisis. These all address the barriers that directly lead to chronic absenteeism.
- TANF offers the block grant to the govt, which then uses them for services like job training, education assistance (like the TAPIT program for college), and youth programs,
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ESSA

- A DCPS school is eligible to become a Title I schoolwide program if the poverty rate of the school is 35% or more. Eligible schools are permitted to use Title I, Part A funds in a combination with state and local resources and other federal education program funds to

enhance the education program of the school and raise the academic achievement of all students. Schools with less than 35% poverty rate are considered Non-Title I.

- List of DCPS title 1 and non-title 1 schools: https://dcps.dc.gov/publication/list-title-i-and-non-title-i-schools
- Title IVe supports family engagement grants to build the relationships key to getting students to attend school regularly.

Medicaid

- Could be used to pay for the Integrated School-Based Health Centers (SBHCs) and Mobile Health Units
    - DC's pre-existing SBHS do get medicaid reimbursement funding, but its really difficult to find a reference cost.
- SBHCs could bill Medicaid directly as a certified provider, often as a Federally Qualified Health Center (FQHC) or through the school district as an agent of the state.
- Sections 1905(a) and 1903(c) of the Social Security Act permit Medicaid payment for SBHS provided to children under the Individuals with Disabilities Education Act (IDEA) through a child's individualized education plan (IEP).

https://osse.dc.gov/sites/default/files/dc/sites/osse/service_content/attachments/Medicaid%20LEA%20Guidelines%20November%202021docx.pdf. Medicaid guidelines for LEAs

- To be eligible for Medicaid reimbursement for SBHS, an LEA must first apply to DHCF to become a Medicaid provider
- Medicaid covered SBHS in the District of Columbia include: • Audiology; • Behavioral Supports; Psychiatric and psychological services; Counseling services; Social work; Nutrition Services; Occupational Therapy; Orientation and Mobility Services; Physical Therapy; • Psychological Evaluation; Skilled Nursing; Specialized Transportation; and • Speech-Language Pathology
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https://osse.dc.gov/page/esea-title-grants - Elementary and Secondary Education Act (ESEA) Title Grants

- OSSE currently administers $86 million in funding to 59 local education agency (LEA) grantees.
- Title I-A funding can be used for a broad range of activities that improve academic programs, educator quality and student outcomes in a school. Grant programs must be designed to help students attain high academic achievement, and they must be developed by school leadership with input from the school community.
- II-A funding can be used for educator professional development and teacher recruitment and retention activities.
- ESEA, Title III, Part A (Title III-A) grant supports students who are learning English as a second language.
- ESEA, Title IV, Part A (Title IV-A) grant improves schools with LEA-level supports for access to and opportunities for a well-rounded education for all students, school conditions for a safe and healthy learning environment, and the use of instructional technology.

https://osse.dc.gov/sites/default/files/dc/sites/osse/page_content/attachments/FY26%20ESEA%20Title%20Prelim%20Allocations.pdf

- **FY26 ESEA TITLES I-A, II-A, III-A, AND IV-A School Year 2025-2026. Funding allocations by school!**

6.2.2 Federal Funding Sources

ESSER (Elementary and Secondary School Emergency Relief)
- Amount potentially available: $X
- Eligible uses for this proposal
- Timeline constraints

https://osse.dc.gov/page/elementary-and-secondary-school-emergency-relief-esser-grants

- ESSER grants made to the District of Columbia from the United States Department of Education (USED) total more than $600 million and include three separate grants that fund recovery efforts from 2020-2024:

	DC Award	LEA Grants (90%)	SEA Funds (10%)
ESSER I-CARES	$42,006,354.00	$37,805,718.00	$4,200,636.00
ESSER II-CRRSA	$172,013,174.00	$154,811,857.00	$17,201,317.00
ESSER III-ARP	$386,476,999.00	$347,829,299.10	$38,647,699.90
TOTAL	$600,496,527.00	$540,446,874.00	$60,049,653.00

https://osse.dc.gov/page/lea-esser-dashboard

- Dashboard of how LEAs spent ESER funds

Title I Funds
- Current D.C. allocation
- How funds could support this initiative

21st Century Community Learning Centers
- Applicability to extended services and family engagement

HRSA School-Based Health Center Grants
- Capital and operational funding opportunities

Medicaid and CHIP
- Reimbursement for health services
- Administrative claiming opportunities

6.2.3 Philanthropic Partnerships
- Target foundations (education, health, equity focus)
- Types of support (startup costs, evaluation, innovation)
- Cultivation and proposal strategy

6.2.4 Cost Savings and Revenue Protection
- Protected enrollment-based funding from improved attendance
- Reduced long-term remediation and intervention costs
- Economic benefits to D.C. (workforce, reduced social services)

6.3 Return on Investment
- Cost per student served
- Cost per percentage point reduction in CA
- Long-term economic benefits (cite research on costs of dropout, etc.)
- Comparison to cost of inaction

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7. POLITICAL CONSIDERATIONS & STAKEHOLDER ANALYSIS

7.1 Stakeholder Analysis
7.1.1 Key Stakeholders and Positions

Mayor's Office
- Interests: Education outcomes, equity, efficient use of resources
- Expected position: Supportive (aligned with priorities)
- Engagement strategy: Direct briefings, data-driven case

D.C. City Council
- Interests: Constituent concerns, budget oversight, accountability
- Expected position: Variable by member
- Key champions to cultivate: [identify specific members]
- Potential opposition concerns: Cost, oversight
- Engagement strategy: Committee briefings, ward-level data

DCPS Leadership
- Interests: Student outcomes, operational feasibility, resources
- Expected position: Supportive with implementation concerns
- Engagement strategy: Collaborative planning, address capacity concerns

Teachers and School Staff (WTU)
- Interests: Workload, support, professional development

- Expected position: Cautiously supportive
- Potential concerns: Additional responsibilities without support
- Engagement strategy: Union consultation, clear role definitions

Parents and Families
- Interests: Student wellbeing, access to services, respect
- Expected position: Supportive (if culturally responsive)
- Potential concerns: Surveillance, blame, trust
- Engagement strategy: Community listening sessions, parent ambassadors

Community-Based Organizations
- Interests: Partnership opportunities, resources, recognition
- Expected position: Supportive
- Engagement strategy: Early involvement, partnership roles

District Agencies (DHS, CFSA, DHCD, DOH)
- Interests: Mission alignment, resources, coordination burden
- Expected position: Variable
- Engagement strategy: Cross-agency working group, clear MOUs

7.2 Political Opportunities and Challenges

7.2.1 Opportunities

- Post-pandemic attention to attendance as priority
- Mayor's authority over DCPS enables action
- Federal funding availability (ESSER, etc.)
- Equity focus aligns with D.C. values
- Evidence base from other jurisdictions

7.2.2 Challenges

- Budget constraints and competing priorities
- Coordination complexity across agencies
- Potential resistance to new bureaucracy
- Sustainability concerns beyond initial funding
- Political timeline (election cycles)

7.3 Recommendations for Political Strategy

- Build coalition before announcement
- Frame as equity and economic investment
- Start with pilot to demonstrate results before scaling
- Establish bipartisan/broad support on Council
- Engage families and community early
- Prepare for implementation challenges with contingency plans

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8. CONCLUSION & RECOMMENDATIONS (1 page)

8.1 Summary of Key Findings

- Restate the problem: 39.2% CA rate affecting 32,384 students
- Restate root causes: Multi-dimensional requiring comprehensive response
- Restate evidence: Integrated approaches work; D.C. governance enables action
- Restate proposal: Five-pillar framework with phased implementation

8.2 Policy Recommendations

Recommendation 1: Adopt the Five-Pillar Framework
- Issue executive order establishing Office of Equitable Attendance
- Commit to phased implementation beginning with 12-school pilot

Recommendation 2: Allocate Necessary Resources
- Include $X million in FY[XX] budget request
- Pursue federal grants and philanthropic partnerships

Recommendation 3: Establish Cross-Agency Coordination
- Formalize MOUs with DHS, CFSA, DHCD, DOH
- Create inter-agency working group with mayoral authority

Recommendation 4: Invest in Rigorous Evaluation
- Partner with independent evaluator
- Commit to data-driven decision making and transparency

Recommendation 5: Engage Stakeholders Early and Often
- Launch community engagement before implementation
- Establish parent and community advisory structures

8.3 Call to Action

- Urgency: Every day of inaction = more students falling behind
- Opportunity: D.C. can be national leader in addressing CA
- Feasibility: Evidence-based approach with strong governance foundation
- Request: Meeting within 30 days to begin implementation planning

8.4 Closing Statement

- Vision for D.C. students: Every child in school, every day
- Long-term impact: Academic achievement, equity, economic prosperity
- Final call: The time to act is now